

● YELLOW · WATCH CLOSELY

PART 2 · CARE OPTIONS

CHAPTER 6

Home Health

THIS CHAPTER IN 30 SECONDS

- **What it is:** medical care at home — nursing, PT, OT, speech, social work, limited aide visits — ordered by a clinician for people who meet the requirements.
- **What it is not:** all-day supervision, housekeeping, meals, or a sitter. That is private duty care (Chapter 7).
- **When to ask:** after a fall, hospitalization, surgery, new weakness, new wound, medication change, or a decline in bathing, dressing, or walking.
- **How to ask:** not "order home health" — say *"Can we schedule a provider evaluation to determine whether home health is appropriate?"*
- **If the answer is no:** ask what criteria are not met and what support they recommend instead.

WHAT FAMILIES USUALLY THINK

That "home health" means someone stays with Mom — bathing, meals, supervision, every day. It does not. Home health is a specific, intermittent medical service. Valuable, even powerful — but do not build the plan on the wrong promise.

What Home Health May Include

- ☐ **Skilled nursing** — wound care, medication and disease teaching, symptom monitoring
- ☐ **Physical therapy** — strength, balance, walking, transfers, fall risk
- ☐ **Occupational therapy** — bathing, dressing, equipment, home setup for safer independence
- ☐ **Speech therapy** — swallowing, communication, cognition
- ☐ **Medical social work** — resources, planning, caregiver stress
- ☐ **Home health aide** — limited personal care only as part of the skilled plan, not daily caregiving

What It Usually Does Not Provide

All-day or overnight supervision, housekeeping, meals, transportation, companionship, sitting services, or twenty-four-hour care. Home health is rarely the whole plan — when the need is daily support, look at private duty care, adult day care, or a care-setting change.

Home Health vs. Private Duty

HOME HEALTH	PRIVATE DUTY CARE
Medical, intermittent, clinician-ordered	Non-medical daily help, often paid privately
Nursing, therapy, social work, limited aide	Bathing, meals, companionship, supervision
After illness, wounds, weakness, or decline	When the person cannot safely be alone
Not long blocks of supervision	Can cover longer blocks and overnight

A family may need both: home health to strengthen, teach, and treat — private duty for daily support. The question is not which sounds better; it is what your parent actually needs.

HOW TO ASK, THE RIGHT WAY

For Medicare-covered home health, a clinician must find a **skilled need, homebound status**, and complete the required face-to-face documentation. So do not ask "Can you just order home health?"

Ask: "**Can we schedule a provider evaluation to discuss this recent change and determine whether home health is appropriate?**" Bring facts: falls, new weakness, medication concerns, bathing or dressing difficulty, recent hospitalization, wounds, confusion.

"Homebound," in Plain English

Not "never leaves the house." It means leaving home is difficult, taxing, unsafe, or takes help or considerable effort — medical appointments and occasional outings are still allowed. Describe what is happening ("He uses a walker and gets exhausted," "She cannot manage stairs safely") and let the clinician decide.

Follow the Insurance

When my father was referred to home health, a social worker asked, "What insurance does he have?" It sounded routine — it was not. Plans reimburse differently, and behind every referral is a business reality. Do not become cynical; become informed: *Why this agency? What others are available? Does it accept my insurance? Do I have a choice?*

When Home Health Is Not Enough — or Not Ordered

If your parent cannot safely be alone, wanders, needs daily medication supervision, or keeps falling despite therapy, the need is bigger than intermittent visits — think private duty, adult day care, assisted living, memory care, or a palliative/hospice evaluation. And if the clinician says no to home health, ask two questions: "**What criteria are not being met?**" and "**What support do you recommend instead?**" "Not home health" should start the next conversation, not end it.

WHAT TO DO THIS WEEK

1. **Write down what changed** — facts: "Mom fell twice in two weeks."
2. **Ask for a provider evaluation** using the script below.
3. **Bring your notes** — fall log, medication list, discharge papers.
4. **Make the home ready** — show the clinician the bathroom, stairs, and walking paths.

■ WORKSHEET — HOME HEALTH EVALUATION

Use this before a provider visit or home health evaluation.

Person receiving care _____

Main caregiver / phone _____

Primary doctor / provider / phone _____

Preferred home health agency (if any) _____

Insurance _____

What changed? (check all that apply)

- | | |
|---|---|
| ☐ Fall or near fall | ☐ Medication concern |
| ☐ New weakness or trouble walking | ☐ Increased confusion |
| ☐ Trouble transferring | ☐ Shortness of breath or pain |
| ☐ Recent hospitalization or rehab stay | ☐ Difficulty bathing, dressing, or toileting |
| ☐ Recent surgery | ☐ Caregiver unable to manage safely |
| ☐ New wound | |

Current safety concerns

CONCERN	YES / NO / UNSURE	NOTES
Safe walking		
Safe transfers		
Safe bathing & toileting		
Medication safety		
Wound or skin concern		
Breathing / swallowing concern		
Confusion or memory concern		

Safe to be alone		
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Current equipment in the home (check what applies)

- | | |
|---|---|
| ☐ Cane | ☐ Bedside commode |
| ☐ Walker | ☐ Hospital bed |
| ☐ Wheelchair | ☐ Oxygen |
| ☐ Shower chair | ☐ Medical alert device |
| ☐ Grab bars | ☐ Medication organizer |
| ☐ Raised toilet seat | |

If home health starts

Agency name & phone _____

Start of care date _____

Primary nurse / therapist _____

Visit schedule _____

DISCIPLINE	NAME	PHONE
Nurse		
Physical therapist		
Occupational therapist		
Speech therapist		
Social worker		
Home health aide		

■ QUESTIONS FOR THE PROVIDER

1. Does my parent meet criteria for home health, and is there a skilled need?
2. Is my parent considered homebound, and was the required face-to-face encounter completed or scheduled?
3. Would nursing, physical therapy, occupational therapy, speech therapy, or social work help?
4. Would a home health aide be part of the care plan?
5. What documentation is needed?
6. What should we do if home health is not appropriate — what criteria are not being met?

7. What support is needed beyond home health?

■ HOME HEALTH SCRIPT

Calling the provider's office: "My parent has had a recent change in function and safety. We are seeing concerns with _____. Can we schedule an appointment or provider evaluation to determine whether home health is appropriate and whether the required documentation can be completed if they meet criteria?"

At the visit: "Based on what you are seeing today, does my parent meet criteria for home health services such as nursing, PT, OT, speech, social work, or aide support as part of the care plan?"

If the answer is no: "What criteria are not being met, and what support would you recommend instead?"

ROB'S NOTE

I have seen families disappointed because they expected home health to cook meals, supervise dementia, and take over the whole day. And I have seen families miss its real value: the doctor's office sees a patient in a chair for fifteen minutes — home health sees the narrow hallway, the bathroom without grab bars, the pill bottles on the counter, the stairs that have become a mountain.

Home health is not the whole answer. But sometimes it is the first trained set of eyes that helps a family understand what the next answer should be.

CHAPTER 6 TAKEAWAY

Home health is medical care in the home for people who meet the requirements — not private duty care, housekeeping, or all-day supervision. The next right question is: **can we schedule a provider evaluation to determine whether home health is appropriate, and what support is needed if it is not?**